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Case Number: 23SMCV04063 Hearing Date: September 17, 2026 Dept: M

CASE NAME: Sabia, et al.,

v. Rubin, et al.

CASE NO.: 23SMCV04063

MOTION: Motion

for Summary Judgment

HEARING DATE: 9/17/2026

Legal

Standard

A party may move for summary judgment in any action or proceeding if it is contended the action has no merit or that there is no defense to the action or proceeding. (CCP, § 437c(a).) "The purpose of the law of summary judgment is to provide courts with a mechanism to cut through the parties' pleadings in order to determine whether, despite their allegations, trial is in fact necessary to resolve their dispute." (Aguilar v. Atlantic Richfield Co. (2001) 25 Cal.4th 826, 843.)

"A party may move for summary adjudication as to one or more causes of action within an action, one or more affirmative defenses, one or more claims for damages, or one or more issues of duty, if the party contends that the cause of action has no merit, that there is no affirmative defense to the cause of action, that there is no merit to an affirmative defense as to any cause of action, that there is no merit to a claim for damages, as specified in Section 3294 of the Civil Code, or that one or more defendants

either owed or did not owe a duty to the plaintiff or plaintiffs.” (CCP, § 437c(f)(1).) If a party seeks summary adjudication as an alternative to a request for summary judgment, the request must be clearly made in the notice of the motion. (*Gonzales v. Superior Court* (1987) 189 Cal.App.3d 1542, 1544.) “[A] party may move for summary adjudication of a legal issue or a claim for damages other than punitive damages that does not completely dispose of a cause of action, affirmative defense, or issue of duty pursuant to” subdivision (t). (CCP, § 437c(t).)

To prevail, the evidence submitted must show there is no triable issue as to any material fact and that the moving party is entitled to judgment as a matter of law. (CCP, § 437c(c).) The motion cannot succeed unless the evidence leaves no room for conflicting inferences as to material facts; the court has no power to weigh one inference against another or against other evidence. (*Murillo v. Rite Stuff Food Inc.* (1998) 65 Cal.App.4th 833, 841.) In determining whether the facts give rise to a triable issue of material fact, “[a]ll doubts as to whether any material, triable, issues of fact exist are to be resolved in favor of the party opposing summary judgment...” (*Gold v. Weissman* (2004) 114 Cal.App.4th 1195, 1198-99.) “In other words, the facts alleged in the evidence of the party opposing summary judgment and the reasonable inferences there from must be accepted as true.” (*Jackson v. County of Los Angeles* (1997) 60 Cal.App.4th 171, 179.) However, if adjudication is otherwise proper the motion “may not be denied on grounds of credibility,” except when a material fact is the witness’s state of mind and “that fact is sought to be established solely by the [witness’s] affirmation thereof.” (CCP, § 437c(e).)

Once the moving party has met their burden, the burden shifts to the opposing party “to show that a triable issue of one or more material facts exists as to that cause of action or a defense thereto.” (CCP § 437c(p)(1).) “[T]here is no obligation on the opposing party... to establish anything by affidavit unless and until the moving party has by affidavit stated facts establishing every element... necessary to sustain a judgment in his favor.” (*Consumer Cause, Inc. v. SmileCare* (2001) 91 Cal.App.4th 454, 468.)

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“The pleadings play a key role in a summary judgment motion. The function of the pleadings in a motion for summary judgment is to

delimit the scope of the issues and to frame the outer measure of materiality in a summary judgment proceeding.” (Hutton v. Fidelity National Title Co. (2013) 213 Cal.App.4th 486, 493, quotations and citations omitted.) “Accordingly, the burden of a defendant moving for summary judgment only requires that he or she negate plaintiff’s theories of liability as alleged in the complaint; that is, a moving party need not refute liability on some theoretical possibility not included in the pleadings.” (Ibid.)

Analysis

Defendant Alyssa B. Zackler M.D.

moves for summary judgment against Plaintiffs Meryem Sabia and Raed Odem’s First Amended Complaint (“FAC”). The FAC states two causes of action for medical negligence and loss of consortium.

The elements of medical malpractice

are: (1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) a breach of that duty; (3) a proximate causal connection between the negligent conduct and the resulting injury; and (4) actual loss or damage resulting from the professional’s negligence.” (Simmons v. West Covina Medical Clinic (1989) 212 Cal.App.3d 696, 701-02 citations omitted.) “Both the standard of care and defendants’ breach must normally be established by expert testimony in a medical malpractice case.” (Avivi v. Centro Medico Urgente Medical Center (2008) 159 Cal. App. 4th 463, 467.)

A defendant moving for summary

judgment in a medical malpractice action must “present evidence that would preclude a reasonable trier of fact from finding it was more likely than not that their treatment fell below the standard of care.” (Johnson v. Superior Court (2006) 143 Cal.App.4th 297, 305.) “Both the standard of care and defendants’ breach must normally be established by expert testimony in a medical malpractice case.” (Avivi v. Centro Medico Urgente Medical Center (2008) 159 Cal. App. 4th 463, 467.) “When a defendant moves for summary judgment and supports his motion with expert declarations that his conduct fell within the community standard of care, he is entitled to summary judgment unless the plaintiff comes forward with conflicting expert evidence.” (Munro v. Regents of University of California (1989) 215 Cal. App. 3d 977, 984-985.) An expert declaration, if uncontradicted, is conclusive proof as to the prevailing standard of care and the propriety of the particular conduct of

the health care provider. (Starr v. Mooslin (1971) 14 Cal. App. 3d 988, 999.)

A married person may recover for loss of consortium when his or her spouse has been injured by the negligence of a third party. (Zwicker v. Altamont Emergency Room Physicians Medical Group (2002) 98 Cal.App.4th 26, 31.) The elements for a loss of consortium cause of action are: (1) a valid and lawful marriage between the plaintiff and the person injured at the time of the injury; (2) a tortious injury to the plaintiff's spouse; (3) loss of consortium suffered by the plaintiff; and (4) the loss was proximately caused by the defendant's act. (Vanhooser v. Superior Court (2012) 206 Cal.App.4th 921, 927.) "A cause of action for loss of consortium is, by its nature, dependent on the existence of a cause of action for tortious injury to a spouse." (Id.)

In her motion, Defendant presents expert opinion testimony through the declaration of expert Joel Kessler, M.D., that Dr. Zackler's care and treatment of plaintiff always complied with the applicable standard of care. As such, Defendant presents undisputed evidence that she met the standard of care and that no act of hers harmed Plaintiffs to a reasonable degree of medical certainty.

Defendant first provides the complete background of Sabia's treatment. (UMF 1-22.) Defendant provides foundation for Dr. Kessler's expertise. (UMF 23.) Based on a review of the record, Dr. Kessler opines that Dr. Zackler was well within the standard of care in her evaluation, care, and treatment of plaintiff. (UMF 24.) In sum, Plaintiff was admitted to Henry Mayo Newhall Hospital on October 7, 2022 following persistent flu like symptoms which pointed to infection. (UMF 25.) Dr. Zackler thoroughly evaluated plaintiff and evaluated the imaging that had been performed up to that point. (Id.) She was placed on antibiotics three antibiotics for aggressive medical management. (Id.) She continued to evaluate plaintiff from October 7, 2022 to October 10, 2022 and called for consultations by infectious disease, gynecologic surgery, internal medicine, interventional radiology, and obstetrics and gynecology. (Id.) These consults were made to definitively diagnose plaintiff's complaint, obtain imaging, and begin the process of any surgical intervention to treat the abscess. (Id.) This was within the standard of care since surgical interventions and infectious disease management are best treated by specialists. (Id.) Nonetheless, Dr. Zackler began plaintiff on ceftriaxone, doxycycline, and metronidazole (flagyl) to treat the infection and

abscess. (Id.) It was within the standard of care to manage plaintiff's condition with antibiotics before opting for or proceeding with surgical management. (Id.) Dr. Zackler started plaintiff on three different antibiotics to aggressively tackle the infection. (Id.) Her choice of antibiotic treatment was appropriate. (Id.) Triple antibiotic therapy was necessary here as an initial treatment in order to treat the infection, inflammation, and swelling in plaintiff's abdomen. (Id.) Only if this failed, would surgery be indicated. (Id.) The standard of care dictates 72 hours of antibiotic treatment prior to proceeding with any surgical intervention. (Id.) It was necessary to reduce plaintiff's symptoms from her sepsis before surgical intervention was initiated, otherwise it could have resulted in significant complications including overwhelming sepsis and possible death. (Id.) Immediate surgery was not indicated, rather antibiotic treatment was necessary, warranted, and essential. Plaintiff needed to be medically stabilized first prior to surgery. (Id.)

To a reasonable degree of medical probability, Dr. Kessler further opines that nothing Dr. Zackler did or did not do caused or contributed to plaintiff's injuries. (UMF 26.) Dr. Zackler's initiation of initial antibiotic treatment was warranted before proceeding with surgical treatment. (Id.) The nature of Plaintiff's infection was severe upon her admission. (Id.) Rushing to surgical intervention would have carried unnecessary significant risks including a probability of overwhelming sepsis and possible death. (Id.) Hence, Dr. Zackler ordering antibiotics and exploring initial conservative methods of treatment was within the standard of care and appropriate. (Id.) Dr. Zackler's refusal to rush to surgery was appropriate and did not cause or contribute to plaintiff's alleged injuries in any way. (Id.) Rather, it prevented further harm. (Id.)

The above evidence meets Defendant's initial burden of production. Plaintiffs have the burden present expert testimony that Dr. Zackler's treatment fell below the standard of care. Plaintiffs have not opposed and therefore fail to meet this burden.

Accordingly, the motion is GRANTED.